4.4 Myocardial Infarction

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "The chest pain is crushing and goes to my left arm." • Gives the pain 9/10 **Objective** • Sweaty and pale • ST elevation on ECG • Troponin raised	**ACTUAL** **Acute pain** related to myocardial ischemia and necrosis as evidenced by crushing chest pain, diaphoresis and ECG changes.	Pain relieved within 30 minutes of treatment.	• Assess the pain and take an ECG straight away. • Give aspirin, nitroglycerin and IV morphine as prescribed. • If SpO_2 is below target, give oxygen. • Get ready for reperfusion (PCI or thrombolysis) as ordered. • Keep him on bed rest somewhere calm.	Pain relieved: **goal met**.
Subjective • "I feel weak and dizzy." **Objective** • BP 94/60 mmHg, HR 112/min • Skin cool • Passing less urine	**ACTUAL** **Decreased cardiac output** related to impaired contractility as evidenced by hypotension, tachycardia and cool skin.	Systolic BP stays above 90 mmHg and urine output is at least 0.5 mL/kg/hour.	• Continuous ECG, and vital signs every 15 minutes at first. • Record urine output every hour. • Listen to the heart and lungs for any new murmur or crackles. • Give medications as prescribed.	Hemodynamics stable: **goal met**.
Subjective • *No specific complaint* **Objective** • Acute myocardial injury	**RISK** **Risk for decreased cardiac output** as evidenced by risk factors: dysrhythmias after myocardial infarction.	Any dysrhythmia is picked up and treated quickly.	• Have the defibrillator and emergency drugs ready. • Follow electrolytes, potassium and magnesium in particular. • Report any new dysrhythmia at once.	No untreated dysrhythmia: **goal met**.